

DENTFLUENCE · GO-LIVE · MODULE 1 OF 18

Rollout Plan — Daily Huddle

How to actually get this screen into your clinic's morning, in four steps over three weeks. Two lanes: what you do, and what the team does. Nothing here needs a developer except the two fixes at the top.

■ Your lane — decisions, setup, watching ■ Team lane — the daily habit

Two things must be true before you start

1 · The background service is running. Tasks from lab overdues, purchase orders, recalls and practice protocols only appear on the huddle board if the scheduled job service is alive on the server. If it is down, the Tasks and Lab columns will look deceptively quiet and nobody will know why. Ask for a confirmation that the *scheduler* and *queue* services are up before the first huddle — not after.

2 · The fake Marketing column is gone and the broken link is hidden. *Both decided 28 August — remove the column now, hide the link now.* Train people on a board that shows one invented column and one error message and they will quietly distrust the seven real columns too. Roughly a day of work between them; it must land before anyone is trained, not after.

What your answers changed. The habit this rollout is built around has moved. It is no longer the front desk filling "amount to collect" at booking — it is **the manager filling it during the huddle itself**, one click per patient on the board. That is a better fit for a ten-minute standing meeting, and it needs no development: the edit card already exists. Every step below has been rewritten around it.

Week -3

Decide and prepare

Nothing changes in the clinic this week. You are removing every reason the habit could fail later.

YOU

- ☐ **Get the two fixes done.**
Remove the Marketing column, hide "View All Yesterday". One day of work. Decided 28 Aug.
- ☐ **Nothing left to decide on this module.**
N2 — projected vs actual collections — was dropped to V1.1 on 28 Aug. Every other question on the Daily Huddle is closed. The manager still fills the collections figure every morning: that habit is what makes the comparison possible whenever it does get built.
- ☐ **Set the huddle time and protect it.**
Pick a time 15 minutes before your first regular appointment. Put it in everyone's shift. A moving huddle becomes no huddle. Allow 12 minutes, not 10 — the manager now fills collections inside it.
- ☐ **Decide who leads it, and confirm the manager attends every day.**
The lead can be your front desk lead. But the collections step is the manager's, so the huddle now has two named roles, not one. Agree who covers the manager's part when they are away.
- ☐ **Turn on the Daily Huddle permission for everyone who attends.**
Settings → Staff & Roles. Everyone needs *View*; whoever ticks tasks off needs *Edit*.
- ☐ **Sort out the screen.**
A wall-mounted monitor or TV near reception. On a laptop only one person reads it — and then it is a report, not a huddle.

TEAM

- ☐ **Nothing yet.**
Do not announce it this week. Announce a thing that works, once.

Week -2

Fill the board with real data

The huddle only looks useful if the data behind it is being entered. This week is about the habit that feeds it.

YOU

- ☐ **Open the huddle alone, every morning, for five days.**
Read all eight columns yourself. You are looking for empty columns — each empty column is a data-entry habit that does not exist yet.
- ☐ **Try the edit card yourself on three patients.**
Click a card in Today's Patient Flow, fill Amount to Collect, Save. Time it. If it takes you more than fifteen seconds a patient, the manager will not sustain it and we need to know that now, not in week three.
- ☐ **Watch one number: "x of 12 appointments logged" under Collections.**
If it reads 2 of 12, that is your week's problem — and from now on it is a manager habit inside the huddle, not a booking habit at the desk.
- ☐ **Check that yesterday's patients actually appear.**
If Yesterday's Flow is empty on a day you saw patients, visits or consultations are not being recorded — fix that first. It is the most valuable column on the board.
- ☐ **Print the Marathi staff guide, chapter 1.**
One copy at reception, one in the doctor's room.

TEAM

- ☐ **Manager: practise the collections pass once a day.**
Open the huddle alone, go down today's list, click each card, fill Amount to Collect, Save. No meeting yet — just build the muscle so it is fast when the team is standing there.
- ☐ **Record every visit and consultation the same day.**
Not the next morning. Yesterday's Flow reads what was recorded, not what happened.

Week -1

Run it as a rehearsal

Five real huddles with the training wheels on. By Friday your front desk lead runs it and you say nothing.

YOU

- ☐ **Mon–Tue: you lead it.**
Follow the ten steps in the Marathi guide exactly, out loud, in order. You are demonstrating a sequence, not showing software.
- ☐ **Wed–Thu: front desk leads, you prompt.**
Let them get it wrong. Correct only the order, not the wording.
- ☐ **Fri: front desk leads, you stay silent.**
If it works with you saying nothing, it will work when you are not there.
- ☐ **Friday afternoon: open the Weekly Report together.**
Ignore the collections cards until H3 is fixed. Read appointments, new patients, lab and protocol compliance.

TEAM

- ☐ **Everyone stands. Twelve minutes, hard stop.**
A huddle that runs twenty minutes gets cancelled by week three.
- ☐ **Manager fills Amount to Collect for every scheduled patient, in the meeting.**
Not before, not after. Doing it in front of the team is what makes the number a shared commitment rather than an admin task.
- ☐ **Every task gets a name before the huddle ends.**
"Someone will call them" is how the list grows.
- ☐ **Every patient in Yesterday's Flow with no next appointment gets + *Book* or + *Task* — during the huddle.**
Not afterwards. This is the single habit the whole screen exists for.
- ☐ **Say when something on screen is wrong.**
Wrong data is a gift in week one and a disaster in month three.

Go-live week +

Make it boring

Success is when nobody mentions the huddle any more because it simply happens.

YOU

☐ Do not attend every day.

Attend twice a week, unannounced. Your job now is to notice whether it happened, not to run it.

☐ Every Friday, open the Weekly Report.

Five minutes. Look at the four numbers below, not at everything.

☐ At four weeks, decide whether to keep the 10-minute format.

If it has crept to twenty, something else belongs in a different meeting.

TEAM

☐ Huddle happens whether or not the owner is in.

☐ Tasks are ticked the moment they are done, not at day end.

☐ Anything wrong on screen is fixed in its own module, that day.

HOW YOU WILL KNOW IT IS WORKING

Four numbers. Write them down each Friday for the first month — the trend matters far more than the value.

NUMBER	WHERE YOU READ IT	WEEK 1 IS FINE AT	BY WEEK 4 YOU WANT
Huddles actually held	Your own count	4 of 6 days	6 of 6, including days you are away
Appointments with an amount to collect the manager's number	The small line under Collections (Today) — "x of y logged"	Anything above 3 of 10	Every scheduled patient, every day. The manager does this inside the huddle, so a gap means the step was skipped — not that someone forgot at the desk.

NUMBER	WHERE YOU READ IT	WEEK 1 IS FINE AT	BY WEEK 4 YOU WANT
			This is the number that turns the huddle from a briefing into a target
Yesterday's patients left with no next step	Count the cards in Yesterday's Flow still saying "No next appointment" at the end of the huddle	Under half	Zero at the end of every huddle. Not zero on the board — zero left unresolved when the huddle ends
Overdue tasks	The red Overdue badges in the Tasks column	Going up is normal — you are finally seeing them	Trending down for three weeks straight

IF IT STARTS SLIPPING

Three failure patterns, and what each one actually means.

WHAT YOU SEE	WHAT IT REALLY MEANS	WHAT TO DO
The huddle is skipped on busy days	It is scheduled too close to the first patient	Move it five minutes earlier. Do not shorten it
It has grown to twenty minutes	Case discussion has leaked into it	Split them. The huddle is <i>what</i> is happening today; case discussion is <i>how</i> , and belongs elsewhere
Columns are empty and nobody mentions it	The team has stopped believing the board	Find which upstream habit stopped — usually same-day visit recording. Fix that, not the huddle
Only the front desk attends	Doctors do not see anything for themselves in it	Make Yesterday's Flow the first thing read, and read the doctor's own next-action notes aloud

WHAT YOU SEE	WHAT IT REALLY MEANS	WHAT TO DO
The manager stops filling Amount to Collect	Either the huddle is running late by the time it reaches that step, or the manager is not in the room	Move the collections pass earlier — right after the patient list is read, before Yesterday's Flow. It is two minutes and it is the step with the clearest money attached

One line to remember. The huddle does not make the clinic better by being looked at. It makes the clinic better because of one action taken during it — booking or tasking every patient who left yesterday without a next step. If that single habit sticks and nothing else does, the module has paid for itself.

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